

HOW TO OPEN A BRAIN INJURY PERSONAL CARE HOME IN GEORGIA

A Georgia DCH-licensed Personal Care Home (PCH) serving adults with brain injury — residential care, personal services, and support toward greater independence.

A step-by-step startup and operations guide

Licensing (DCH HFRD) · Staff & training · Funding · The program · Accreditation

Regulatory basis: Rule Chapter 111-8-62, Ga. Comp. R. & Regs. (Personal Care Homes); Chapters 111-8-25, 111-8-12, and 111-8-100; O.C.G.A. Title 31. Funding: the Independent Care Waiver Program (ICWP) and the Brain and Spinal Injury Trust Fund.

How to Use This Guide

This guide walks you through opening and operating a Personal Care Home (PCH) in Georgia that serves adults with brain injury. That home is licensed by the Georgia Department of Community Health (DCH), Healthcare Facility Regulation Division (HFRD), under Rule Chapter 111-8-62. There is one licensing path — get the home licensed as a PCH — and a funding picture built around the Independent Care Waiver Program (ICWP), the Brain and Spinal Injury Trust Fund, and private pay. Georgia has no separate brain-injury facility license, so the PCH is your vehicle.

Important — verify before you act

This guide is an educational resource, not legal, financial, tax, or billing advice. Requirements, forms, fees, and rates change. Confirm every step with DCH HFRD and, for funding, with the ICWP program (Alliant Health Solutions) and the resident's payer before you act. A PCH provides personal care and supervision — it does not provide 24-hour skilled nursing; a resident who needs more must be served at an appropriate level of care.

The big picture, in one paragraph

Here is the whole business in a nutshell, so the details ahead have somewhere to land. You will license a home with the State (DCH/HFRD) as a Personal Care Home built for adults with brain injury. You will staff it with screened, trained people and run it on written policies. You will fill it through relationships with rehabilitation hospitals, trauma centers, and waiver case managers. And you will fund it through a blend of the Independent Care Waiver Program (Medicaid), private pay, and the Brain and Spinal Injury Trust Fund — with private pay as a deliberate base because the waiver has a waiting list. Everything in this guide is a detail hanging off one of those four pegs: license it, staff it, fill it, fund it.

The path at a glance — step by step

1. Set up the business — form your Georgia entity, secure the home, and confirm local zoning allows a personal care home at the address.
2. Get the home ready — meet the 111-8-62 design, physical-plant, and fire-safety standards, and line up the local fire inspection.
3. Pass background checks — fingerprint records checks (GCHEXS) for owners, the administrator/on-site manager, and all direct-access staff.
4. Complete training — the 60-day training, the 90-day fire-safety training, and continuing education; set up proxy-caregiver training if unlicensed staff will handle health-maintenance tasks.
5. Write your policies — your 111-8-62 policy manual and the forms it references (required for the application).
6. Apply to DCH HFRD — submit the PCH application online with documents and fee; receive a provisional license.
7. Pass the initial on-site survey — HFRD issues your permit once you demonstrate compliance.
8. Line up funding — enroll as a Georgia Medicaid / ICWP provider and connect with Alliant case managers and referral sources.
9. Admit and document — welcome residents, deliver services, and document to stay survey-ready.

Part 1 — What This Business Is

A Georgia PCH provides housing, food service, and one or more personal services to two or more adults not related to the owner or administrator. For brain-injury residents, it provides supervision, assistance with daily living, and support toward independence. Homes are classified by size, and a PCH serving 25 or more residents is an Assisted Living Community (ALC, Rule 111-8-63).

- **Family PCH:** 2–6 residents.
- **Group PCH:** 7–15 residents.

- **Larger PCH:** 16–24 residents (25+ becomes an ALC, with an administrator licensed by the State Board of Long-Term Care Facility Administrators).

License: Personal Care Home permit, issued by DCH HFRD under Rule 111-8-62.

Why brain injury is its own niche — and why that matters to your business

A brain injury is not the same as aging or a developmental disability, and that difference is the heart of your business. Many of your residents will be younger adults — often 21 to 64 — who were independent before a car crash, fall, assault, stroke, or overdose changed their lives. They may walk and talk normally but struggle with memory, initiation, impulse control, judgment, fatigue, and emotional regulation. The care they need is less about physical nursing and more about structure, cueing, supervision, and patient skill-building. A generic personal care home that treats them like elderly residents will fail them; a home built around brain injury — consistent routines, memory and organization supports, calm redirection, and community re-integration — is genuinely hard to find, which is exactly why families, hospitals, and case managers are looking for you.

What you are actually selling is trust and specialization: a safe, structured home where a person with a brain injury can rebuild daily-living skills and, where possible, move toward greater independence. That specialization lets you stand apart from ordinary personal care homes, build referral relationships with rehabilitation hospitals, and — with accreditation — command stronger payer relationships. The demand is real and under-served: Georgia has thousands of adults living with the long-term effects of traumatic brain injury, a dedicated Medicaid waiver (ICWP) that struggles to find qualified residential providers, and a Trust Fund created specifically because the need outstrips the supply of services.

Part 2 — Who Regulates You

- **DCH HFRD** — Department of Community Health, Healthcare Facility Regulation Division. Licenses your PCH, sets the 111-8-62 standards, and conducts the licensure survey.
- **DCH Medicaid / ICWP** — administers the Independent Care Waiver Program that funds services for eligible TBI residents; screening and case management run through Alliant Health Solutions.
- **Brain and Spinal Injury Trust Fund Commission** — administers flexible, payer-of-last-resort funding for Georgians with brain and spinal cord injury.
- **Adult Protective Services & law enforcement** — receive reports of abuse, neglect, or exploitation under Georgia's Long-Term Care Resident Abuse Reporting Act.

Here is how they fit together in practice: HFRD is your licensing regulator — it decides whether you can open, inspects you, and can cite or fine you. DCH Medicaid and ICWP are your payer side — separate from licensing; being licensed does not automatically make you a Medicaid provider, and being a Medicaid provider does not license your home. You deal with both, but through different offices and different applications. The Trust Fund is a supplemental, flexible funder you tap for specific resident needs, not a primary payer. And the Ombudsman and Adult Protective Services are oversight bodies you will interact with around resident rights and safety. Knowing which office owns which question saves you weeks of being bounced around — which is exactly why the contact list near the end of this guide is organized by 'what you need' rather than by agency name.

Part 3 — Business and Insurance Setup

- Form your business entity with the Georgia Secretary of State and obtain any required local business license.
- Secure the home and confirm local zoning permits a personal care home at the address (county/city requirements vary).
- Obtain liability and property insurance appropriate to a residential care home.
- Identify all owners and controlling interests (10%+) — they and staff complete fingerprint records checks.
- Obtain a National Provider Identifier (NPI) and plan Georgia Medicaid enrollment if you will bill ICWP.

What each of these really means

The entity is your legal shield: most operators form an LLC or corporation so that the business — not you personally — holds the license, signs the lease, and carries the liability. Keep the business's money and your personal money completely separate from day one; commingling is one of the fastest ways to lose the legal protection you set the entity up to get. Register with the Georgia Secretary of State, get an EIN from the IRS, and open a business bank account before you spend a dollar on the home.

Zoning is the step most first-time operators underestimate. Before you sign a lease or buy, confirm in writing that a personal care home is an allowed use at the specific address — rules differ by city and county, and a home that's fine on one street may be prohibited a mile away. The good news: homes for people with disabilities have protections under the federal Fair Housing Act, so a blanket 'no group homes' answer is often not the final word — but you want this settled before money is at risk, not after. Georgia's community-living siting rules can also apply.

Insurance is not optional and not one policy. At minimum, plan for general liability, professional (or 'abuse and molestation') liability, property coverage on the building and contents, workers' compensation once you have employees, and commercial auto if you transport residents. Tell the insurer plainly that you serve adults with brain injury in a licensed personal care home — underpricing the risk by describing it as ordinary housing will leave you uncovered when it matters. Finally, the NPI and Medicaid enrollment are what let you bill ICWP later; you can start the NPI early (it's free) even before the home opens.

Part 4 — The Home: Physical Plant and Fire Safety

Your home must meet the design, physical-plant, and furnishings standards of Rules 111-8-62-.12, .13, and .14, plus local fire-safety requirements. Before your licensure survey, confirm:

- ☐ The home meets the 111-8-62-.12/.13/.14 design, physical-plant, and furnishings standards, with accessibility for residents with mobility or cognitive impairments.
- ☐ Sleeping-room occupancy is within limits, with required living, dining, and bathing space.
- ☐ Fire-safety systems and a local or state fire-safety inspection, as your county/city requires.
- ☐ An emergency preparedness plan (and, for 25+ beds, the Chapter 111-8-16 disaster-preparedness requirements).
- ☐ Safe hot-water temperatures and secure storage of hazardous items.
- ☐ The home's name displayed so it is visible from the street.

What the survey actually looks at — and where homes lose points

The physical-plant survey is concrete and predictable, which means it is winnable if you prepare. Surveyors check bedroom square footage and occupancy (rooms cannot be over-crowded), that residents have appropriate beds and storage, that bathrooms are sufficient for the number of residents, that hot-water temperature is safe (not scalding), that exits are clear and operable, that the kitchen is clean and food is stored and dated properly, that hazardous chemicals and any residents' medications are stored securely, and that the home is clean, in good repair, and pest-free.

The deficiencies that most often delay a first license are small and avoidable: missing or expired fire extinguisher tags, a water heater set too hot, no documented fire drills, blocked or double-keyed exits, missing GFCI outlets near water, an incomplete first-aid kit, and gaps in required postings. Walk your home with the eyes of a surveyor a week before your survey, fix the small things, and photograph your fire-safety equipment and drills so you can prove compliance on the day. For brain-injury residents specifically, think through wandering and safety: door and exit awareness, secure storage of anything harmful, and a calm, low-clutter environment that reduces confusion and falls.

Part 5 — Step 1: Administrator, Staff, and Required Training

- **Administrator:** the home operates under a qualified administrator or on-site manager, with an authorized alternate. A home of 25+ beds needs an administrator licensed by the State Board of Long-Term Care Facility Administrators.
- **Fingerprint background checks:** owners, the administrator/on-site manager, and all direct-access employees obtain a satisfactory fingerprint records-check determination (Chapter 111-8-12) through GCHEXS / Georgia Applicant Processing Services before direct-access work.
- **Training:** staff complete CPR, first aid, emergency procedures, resident-population needs, residents' rights, abuse reporting, and infection control within 60 days; at least five hours of fire-safety training within 90 days; and direct care staff complete 24 hours of continuing education the first year and 16 hours each year after — including brain-injury and cognitive-care content.
- **Proxy caregivers:** any health-maintenance tasks (such as certain medication administration) by unlicensed staff are performed only by trained, validated proxy caregivers under Chapter 111-8-100.

The fingerprint check, step by step

Background screening in Georgia is a fingerprint-based records check, not a name check, and it runs through the state's system (GCHEXS / Georgia Applicant Processing Services). Each owner, the administrator or on-site manager, and every direct-access employee must have a satisfactory determination before they work with residents — so build the time for it into your hiring. Register the person, have them complete Live Scan fingerprinting at an approved location, and keep the determination in the personnel file. A disqualifying record does not always end the matter — there is a records-check and, in some cases, an exemption process — but no one with an unsatisfactory determination may be in a direct-access role in the meantime.

The training your staff actually need

Georgia is specific about training, and surveyors check it. Within the first 60 days, every staff member completes CPR and first aid, emergency procedures, the medical and social needs and characteristics of your resident population, residents' rights, the Long-Term Care Resident Abuse Reporting Act, and infection control. Within 90 days, staff complete at least five hours of fire-safety training (with a two-hour refresher every three years). Direct care staff then complete at least 24 hours of continuing education in the first year and 16 hours every year after. Build a training calendar and a signed record for each person — 'we trained them' without documentation counts as not trained on survey day.

For a brain-injury home, go beyond the minimum with content your competitors skip: how brain injury affects memory, attention, and behavior; cueing and compensatory strategies; recognizing and de-escalating agitation; seizure awareness; fall prevention; and each resident's individual support strategies so care is consistent across shifts. This training is also a selling point — referral sources and payers want to know your staff are prepared for this population, and accreditation will expect it.

Proxy caregivers — Georgia's answer to 'who can give the medications'

Unlicensed staff cannot freely perform tasks that would normally require a nurse. Georgia's Proxy Caregiver rule (Chapter 111-8-100) is the bridge: with the resident's written authorization and documented, task-specific training and competency validation, an unlicensed 'proxy caregiver' may perform defined health-maintenance activities such as certain medication administration. Treat this seriously — train to the specific task, validate competency before the staff member performs it, keep the records, and re-validate on schedule. It is both a compliance requirement and a resident-safety practice.

Part 6 — Step 2: Get Your PCH Permit from DCH HFRD

1. Download the Personal Care Home application packet and rules from dch.georgia.gov and submit your application online through the HFRD portal.

2. Include the required documents: entity and ownership disclosures, the home's sketch/plat, your policies and procedures (111-8-62), the Records Check Application (Form 5579), and local zoning/fire documentation.
3. **Pay the permit fee.** Confirm the current PCH permit fee on DCH HFRD's licensing-fees page — fees are non-refundable and vary by facility type/size.
4. HFRD issues a provisional license; be ready to begin operating so the initial on-site survey can be scheduled before the provisional period ends.
5. Pass the initial on-site survey; HFRD issues your permit once compliance is demonstrated. A valid permit is required before you operate.

Where your manual and forms fit. The application requires complete 111-8-62 policies and procedures. Your Policy & Procedure Manual and Forms Packet are built to that requirement — have them ready before you apply.

What the initial survey is really like

After you submit and are issued a provisional license, HFRD schedules an initial on-site survey. Expect the surveyor to tour the home, review your policies and personnel files (qualifications, background checks, training), check your physical plant and fire safety, and confirm your admission, medication, and resident-record systems match what your policies say. The survey is not a pop quiz on trivia — it is a check that the home you described on paper actually exists and runs the way your manual promises. If you have real policies, trained and screened staff, a safe building, and organized records, you are in good shape.

If the surveyor finds deficiencies, you are typically given the chance to submit a plan of correction — a written statement of what you will fix, how, and by when — rather than being denied outright. Take it seriously, correct the root cause (not just the single instance), and document the fix. Homes that treat the plan of correction as a genuine improvement, not a paperwork exercise, clear it quickly. Your permit is issued once compliance is demonstrated, and only then may you serve residents.

Plan for months, not weeks. Between preparing the home, screening and training staff, writing policies, the application, the provisional period, and the survey, a realistic first-license timeline runs several months — longer if a plan of correction is needed. Build that runway into your budget so you are not paying rent on an empty, unlicensed home longer than you planned.

Part 7 — Step 3: How You Get Paid — Funding the Home

Here is the honest funding picture. Georgia has no single published PCH per-diem for brain injury; you build funding from these sources:

Source	What it pays / how	Code / rate reality
ICWP (Medicaid waiver, DCH)	Georgia's TBI waiver for adults 21–64 at nursing-facility/hospital level of care. Billable residential services include Personal Support Services (Level 1 & 2) and Alternative Living Service. You enroll as a Georgia Medicaid ICWP provider and bill through GAMMIS. Waiting list; case management via Alliant Health Solutions.	Procedure codes & rates are on the GAMMIS ICWP fee schedule (no single public per-diem). Provider prerequisite: 1 year providing personal support / community living support in CCSP, SOURCE, NOW, or COMP.
Brain & Spinal Injury Trust Fund	Flexible, payer-of-last-resort funding for TBI/SCI post-acute needs (services, equipment, supports); funded by a surcharge on DUI fines and administered by the BSITF Commission.	Application-based; not a fee schedule.
Private pay	Resident or family pays for room, board, and personal care under the admission agreement.	Set by your rate schedule.

The key to know: ICWP is the dedicated brain-injury funder, but it has a waiting list and provider prerequisites, and its billable residential services (Personal Support Services Level 1 & 2, and Alternative Living Service) are billed to Georgia Medicaid through GAMMIS at the rates on the ICWP fee schedule — there is no single per-diem to quote. Plan for private pay and the Brain and Spinal Injury Trust Fund alongside ICWP, and confirm each resident's funding before admission.

Verify each resident's funding before admission

ICWP has a waiting list and enrollment prerequisites, the Trust Fund is a payer of last resort, and Medicaid income limits apply. Confirm each resident's specific coverage — and what it will and will not pay — before you admit.

The ICWP reality — read this before you bank on it

ICWP is the dedicated brain-injury funder, and it is worth pursuing — but go in clear-eyed. First, it has a waiting list: the number of funded slots is limited, so an eligible resident may not be able to start on ICWP immediately. Second, becoming an ICWP provider has a prerequisite that surprises people — the program generally expects a provider to have already delivered personal-support or community-living-support services in another Georgia program (such as CCSP, SOURCE, NOW, or COMP) for about a year. That means the fastest path into ICWP for a brand-new operator is often to first enroll and gain experience in one of those programs, then add ICWP. Third, ICWP pays through Georgia Medicaid's GAMMIS system in 15-minute units for services like Personal Support Services, so your revenue from a given resident tracks the hours authorized in their plan — not a flat daily rate. Confirm the current codes and rates on the GAMMIS fee schedule, and coordinate every resident's authorized hours with their case manager at Alliant Health Solutions.

A note on rates: industry summaries put Georgia waiver personal-support pay in the rough range of four to six dollars per 15-minute unit, but that is not an official figure and advocates have long argued ICWP's rate runs low — do not build a budget on it. Pull the current ICWP fee schedule from GAMMIS for the real numbers before you project revenue.

Pricing your private-pay rate

Because ICWP has a waiting list and won't fund every bed, private pay is not a fallback — it is part of your core model, and you should price it deliberately. Build your monthly private-pay rate from your real costs: rent or mortgage, staffing (your largest cost, and higher for brain injury because of the supervision required), food, utilities, insurance, training, supplies, and a margin. Compare your number to what personal care homes and specialized brain-injury settings in your area actually charge, and be honest that a brain-injury home costs more to run than an ordinary one. Put the rate, what it includes, and your refund and rate-change terms clearly in the admission agreement — vague pricing is a top source of family disputes and complaints.

The other money — Trust Fund, VR, veterans, insurance

- **Brain & Spinal Injury Trust Fund:** flexible, payer-of-last-resort funding for specific TBI/SCI needs — equipment, services, supports — that other sources won't cover. Not a room-and-board payer, but valuable for filling gaps for individual residents.
- **Georgia Vocational Rehabilitation Agency (GVRA):** for residents working toward employment, VR can fund work-reentry services that complement your program.
- **Veterans:** residents who are veterans may have VA benefits; the Charlie Norwood VA (Augusta) and Atlanta VA are both referral and, in some cases, funding connections.
- **Auto & workers' comp:** because many brain injuries are accident-related, a resident may have auto-insurance or workers'-compensation coverage for care — always ask.

Part 8 — Staffing and the Care Team

- Maintain at least the minimum on-site staffing required by Rule 111-8-62-.10, and staff above it for residents' acuity — with awake overnight coverage where residents' needs or bed count require it.

- For brain-injury residents, coordinate with the resident's therapists, providers, and ICWP case manager as a care team.
- Screen (fingerprint), train, and validate all staff, and document competency — not just attendance.

Staffing is your biggest cost and your biggest operational challenge, and brain injury raises the bar: these residents need more supervision and more consistency than a typical personal care home provides, so plan your ratios and your overnight coverage around actual acuity, not just the rule minimum. Consistency matters as much as coverage — a rotating cast of unfamiliar faces is destabilizing for someone with memory and processing difficulties, so invest in retention (fair pay, real training, respect, and a manageable workload) because turnover is expensive in both money and resident wellbeing. Think of yourself as running a care team, not just a staff roster: the resident's therapists, physicians, behavioral providers, and ICWP case manager are all part of it, and your job is to coordinate them around each resident's plan and communicate changes quickly.

Part 9 — The Program and Accreditation

Beyond housing and personal care, a strong brain-injury PCH assesses each resident, builds an individualized support plan with community-integration goals, uses brain-injury-informed cueing and behavioral supports, and plans community re-entry — tracking outcomes over time.

In practice, that means: a real assessment at admission of each resident's cognitive, physical, behavioral, and daily-living abilities and needs, refreshed when things change; an individualized support plan with concrete, measurable goals (for example, managing a medication routine with fewer prompts, or preparing a simple meal independently) developed with the resident and coordinated with their therapists and ICWP case manager; and a predictable daily structure — consistent routines, visual schedules, memory and organization supports — because structure is medicine for a brain injury. Your staff use positive, least-restrictive behavioral supports and trained de-escalation rather than confrontation, and every resident's specific strategies are written down so care is the same on every shift.

Community integration is the part that sets a strong home apart: supervised outings, practicing real-world skills (shopping, transit, appointments), and, where a resident's goals include it, connecting to vocational rehabilitation or volunteer work. And because payers and accreditors increasingly ask 'does your program actually work,' track outcomes at the program level — functional gains, length of stay, where residents go when they leave — not just individual notes. That data becomes your proof of value when you talk to referral sources and payers.

Accreditation — a real Georgia advantage. Payers often prefer national rehabilitation accreditation (the Joint Commission or CARF, which specializes in brain-injury programs) for brain-injury providers. In Georgia there's an added benefit: an accreditation recognized by the Department may support "deemed status" in lieu of routine inspection (Rule 111-8-62-.31). Building your program to align with these frameworks — without reproducing their proprietary standards — strengthens survey readiness, payer contracting, and your inspection standing. Your Policy & Procedure Manual is structured to support this alignment.

Part 10 — Timeline, Costs, and Two Worked Examples

Rough sequence

Plan for several months from start to first billable day. Form your entity and secure/prepare the home → hire and train your administrator and staff (fingerprint checks; required training) → submit the PCH application online with policies, documents, and fee → receive a provisional license → pass the initial on-site survey → receive your permit → enroll as an ICWP/Medicaid provider and line up funding → begin admissions. Survey scheduling and any plan of correction affect the total time; verify current timeframes with HFRD.

Typical startup costs to budget

- The home (lease or purchase), accessibility modifications, furnishings, and fire-safety systems.
- Staff training, fingerprint checks, CPR/first aid; insurance and business setup.

- The DCH permit fee (confirm current) and any local fire/zoning/business-license costs.
- Your Policy & Procedure Manual and Forms Packet.

Budget realistically and keep a cash reserve. Your two largest realities are (1) the pre-revenue runway — months of rent, staffing, and setup before your first billable resident — and (2) staffing, which will be your biggest ongoing cost once you open, higher than an ordinary home because brain-injury care is supervision-intensive. Add the home and its modifications, furnishings, fire-safety and accessibility work, insurance, training and background checks, the DCH permit fee and local permit/business-license costs, and your policy manual and forms. Then add a reserve on top: the operators who make it are the ones who planned for the home to run partly empty for a while, not the ones who assumed full census on day one. Undercapitalization — not lack of demand — is what closes new homes.

Worked example A — a 6-bed group PCH in the Atlanta metro

A single-family home is licensed as a 6-bed group PCH serving adults with brain injury. It takes referrals from Atlanta-area rehabilitation and trauma programs — the Shepherd Center (a nationally recognized catastrophic-injury rehab hospital) and Grady's Marcus Trauma Center are primary sources — and from ICWP case managers at Alliant Health Solutions. Its funding is a mix: residents approved for ICWP have their Personal Support Services and, where authorized, Alternative Living Service billed to Georgia Medicaid through GAMMIS at the ICWP fee-schedule rate (billed in 15-minute units, so revenue tracks each resident's authorized hours); other residents are private-pay under the admission agreement; and the Brain and Spinal Injury Trust Fund can fill specific gaps as a payer of last resort. Because ICWP carries a waiting list, the home budgets a private-pay base rather than assuming every bed is ICWP-funded, and staffs to meet residents' supervision needs and the minimum ratios of Rule 111-8-62-.10. Confirm current ICWP rates on the GAMMIS fee schedule before projecting revenue.

Worked example B — a family PCH in the Augusta area

A smaller family PCH (2–6 residents) in the Augusta area runs the same model on a smaller scale. It draws referrals from the region's Level I trauma center and rehabilitation programs (such as Augusta University Medical Center) and the Charlie Norwood VA, plus ICWP case managers. Given ICWP's waiting list, it leans more on private pay and adds ICWP-funded residents as authorizations come through. The lower bed count means lower fixed costs and allows a solo administrator/on-site manager — no separately licensed administrator is required under 25 beds — but the licensing, screening, training, and billing steps are identical to the Atlanta home. The Trust Fund and Georgia Vocational Rehabilitation can support specific residents' needs.

In both markets, discharge planners at rehabilitation hospitals and trauma centers, and ICWP case managers, are your primary referral sources, alongside families seeking specialized brain-injury residential care. Actual revenue depends on each resident's authorized ICWP services and your private-pay rate; confirm current figures with HFRD, the ICWP program, and the GAMMIS fee schedule.

Part 11 — Common Mistakes That Delay or Sink a Personal Care Home

Most homes that stumble don't fail on the big things — they fail on avoidable ones. Learn these before they cost you.

- **Signing a lease before confirming zoning.** Get written confirmation that a personal care home is allowed at the exact address first. This is the single most expensive mistake.
- **Under-budgeting the pre-revenue runway.** You pay rent, staff, and setup for months before your first billable resident. Homes that run out of cash during licensing don't open.
- **Treating training and files as paperwork.** Missing background checks, undocumented training, and incomplete resident records are the deficiencies that most often delay a first license.
- **Admitting residents you can't safely serve.** A resident whose needs exceed a personal care home (heavy medical or nursing needs) puts them at risk and you out of compliance. Assess honestly and refer up when needed.

- **Assuming ICWP will fund every bed.** The waiting list and provider prerequisite are real. Build a private-pay base into your plan.
- **Vague pricing and admission agreements.** Unclear charges, refunds, and rate changes drive family disputes and complaints. Spell it all out in writing.
- **Weak abuse-reporting discipline.** Failing to report immediately — or retaliating against a reporter — is among the most serious violations. Train it, drill it, and never delay for ‘internal review.’
- **Running the home out of your personal bank account.** Commingling funds erases the liability protection your entity was supposed to give you.

Part 12 — Your First 90 Days Operating

Getting the permit is the start line, not the finish. Your first three months set whether the home runs smoothly or lurches from crisis to crisis. Focus here:

- **Document everything from day one.** Assessments, support plans, medication records, incident reports, drills, and training — your first survey after opening will look at what you actually did, not your intentions.
- **Lock in your referral relationships.** Introduce yourself to discharge planners at the Shepherd Center, trauma centers, and rehab units, and to ICWP case managers at Alliant. Referrals come from relationships, and they take time to build — start before you have an empty bed to fill.
- **Get your Medicaid/ICWP enrollment moving.** If ICWP’s prerequisite applies to you, start building the qualifying experience now so the funding path is open when you need it.
- **Build your staff into a team.** Consistency across shifts is everything for brain-injury residents. Hold regular staff communication, keep each resident’s strategies visible, and address turnover fast.
- **Run your drills and reviews for real.** Fire drills, incident reviews, and a monthly self-check against your policies keep you survey-ready and catch problems while they’re small.
- **Track your outcomes from the first resident.** Start your outcome data early — it becomes your proof of value for payers, accreditation, and referral sources.

Part 13 — When They Say “Check Locally”: Who to Call & What to Ask

Some things are handled by the State (DCH / HFRD), some by the ICWP program, and some by your local city or county (zoning, business license, fire). Here is who to call and exactly what to ask, so you get a straight answer instead of getting bounced around. Verify current numbers before you rely on them.

The statewide offices (same for everyone)

For...	Who to contact
Opening & licensing your PCH (application, forms, fee)	DCH HFRD Applications & Waivers — 404-657-5700 · hfrd.applicationswaivers@dch.ga.gov
PCH rules & program questions	DCH HFRD Personal Care Home Program — dch.georgia.gov (HFRD Contacts page)
Fingerprint background checks (GCHEXS)	DCH — VoluntaryBackgroundCheck@dch.ga.gov
ICWP eligibility & applications (your residents)	Alliant Health Solutions — 888-669-7195 · HCBSWaivers@AlliantHealth.org (or GMCF 800-982-0411)
ICWP billing codes & rates	Georgia Medicaid GAMMIS — mmis.georgia.gov (Provider Information → Fee Schedules)
Brain & Spinal Injury Trust Fund / Central Registry	BSITF Commission — georgia.gov (“Brain and Spinal Injury Trust Fund Commission”)
The complaint process (know how it works)	HFRD Complaint Intake — 800-878-6442

For...	Who to contact
Resident advocacy	Long-Term Care Ombudsman — 1-866-552-4464 (Option 5) · georgiaombudsman.org
Reporting abuse, neglect, or exploitation	911 if in immediate danger · Adult Protective Services 1-866-552-4464

The local calls — and exactly what to ask

① City or County Planning / Zoning — “Can I legally run a personal care home here?”

- “I want to open a personal care home — a residence where adults with brain injury live and receive personal care and supervision. Is that an allowed residential use at [address]?”
- “Are there local permits, occupancy limits, or business-license requirements? Can you confirm in writing?”
- If they hesitate: homes for people with disabilities are protected as a residential use under the federal Fair Housing Act — ask them to point to the specific local rule.

② Local Business License / Occupational Tax Office

- “What business/occupational-tax license do I need to operate a personal care home at [address], and what does it cost?”

③ Fire Marshal (city or county)

- “What fire-safety inspection and systems does a personal care home of [size] need here, and can you schedule the inspection I’ll need for my DCH survey?”

Your two-metro call list — fully worked

Most Georgia brain-injury homes open in or near one of these two markets. Each real call list is done for you below; if you’re elsewhere, use them as a template and the finder at the end.

ATLANTA (and metro counties — Fulton, DeKalb, Cobb, Gwinnett)

- **Zoning & permits:** City of Atlanta Office of Buildings (Dept. of City Planning), 55 Trinity Ave SW, Suite 3900 — residential permits 404-330-6906; zoning Planner on Duty 404-330-6145; inspections office at 96 Mitchell St, 404-865-8400. Permits are filed online through the Accela Citizen Access portal.
- **Fire:** City of Atlanta Fire & Life Safety / Office of the Fire Marshal — 470-716-5173; plan review 404-546-7000.
- **Business license:** City of Atlanta Office of Revenue (occupational-tax certificate).
- **Outside city limits:** Fulton County 404-613-1463; DeKalb County Health Dept 404-294-3700 — or your specific city/county.
- **The local quirk:** “metro Atlanta” is many separate cities (Sandy Springs, Decatur, College Park, East Point, Brookhaven...) and counties, each with its own government — confirm which jurisdiction your exact address is in first, and watch for Special Public Interest (SPI) and historic-district overlays that add zoning rules.
- **Referrals:** Shepherd Center; Grady Marcus Trauma Center discharge planning; ICWP case managers (Alliant 888-669-7195).

AUGUSTA (RICHMOND COUNTY)

- **One government:** Augusta is a consolidated city-county government, so one unified jurisdiction handles planning, zoning, permits, and business licenses.
- **Zoning & planning:** Augusta Planning & Development Department, 535 Telfair St, Suite 300; Augusta–Richmond County Planning Commission 706-821-1796.
- **Building permits & business license:** Augusta Planning & Development — Permitting & Licensing / Construction Division, 1803 Marvin Griffin Rd (business licenses & occupation taxes).
- **Fire:** Augusta Fire Marshal’s Office (fire-alarm/sprinkler plans are submitted separately).
- **The local quirk:** a Historic District address needs Historic Preservation Commission approval, and a flood-plain address needs extra review — ask about both.

- **Referrals:** Augusta University Medical Center; Charlie Norwood VA; ICWP case managers (Alliant 888-669-7195).

How to find your own city or county's numbers (if you're elsewhere)

1. Confirm your jurisdiction: search "[your city] GA planning and development" or "[your county] GA planning & zoning." Many addresses are inside a city; unincorporated areas are handled by the county.
2. Ask Planning/Zoning the "allowed residential use" question above — and get the answer in writing.
3. Ask the Fire Marshal what inspection your PCH size needs for the DCH survey.
4. For your residents' funding, call Alliant Health Solutions (ICWP) at 888-669-7195.
5. Not sure who covers you? Call your county's main line and ask for Planning/Zoning.

Core Rules & References

- **Rule Chapter 111-8-62** — Personal Care Homes (the primary licensing rule; sections .01–.34).
- **Chapters 111-8-25, 111-8-12, 111-8-100, 111-8-16** — general licensing & enforcement; criminal background checks; proxy caregivers; disaster preparedness (25+ beds).
- **O.C.G.A. Title 31, Ch. 2 & 7; § 31-8-80 et seq.** — governing statutes and the Long-Term Care Resident Abuse Reporting Act.
- **DCH HFRD** — forms, applications, and licensing fees: dch.georgia.gov (Healthcare Facility Regulation Division).
- **ICWP & Medicaid** — medicaid.georgia.gov; billing codes/rates on the GAMMIS fee schedule (mmis.georgia.gov); applications via Alliant Health Solutions, 888-669-7195.
- **Rules online** — the full text of Chapter 111-8 is at rules.sos.ga.gov.

Key Terms at a Glance

The alphabet soup you'll hear, in plain language:

- **PCH (Personal Care Home):** the license category for your home — housing, meals, and personal services for adults.
- **ALC (Assisted Living Community):** the larger license (25+ beds) with more allowed services and a licensed administrator.
- **DCH / HFRD:** the Georgia Department of Community Health and its Healthcare Facility Regulation Division — your licensing regulator.
- **ICWP (Independent Care Waiver Program):** Georgia's Medicaid waiver that funds services for eligible adults with TBI (and severe physical disabilities) at nursing-facility level of care.
- **GAMMIS:** Georgia's Medicaid billing system, where the ICWP fee schedule (codes and rates) lives.
- **Alliant Health Solutions:** the organization that handles ICWP screening and case management.
- **Proxy caregiver:** a trained, validated unlicensed staff member allowed to perform specific health-maintenance tasks under Rule 111-8-100.
- **Direct-access employee:** anyone with access to a resident or their property, records, or funds — all must pass the fingerprint check.
- **Deemed status:** recognition (under Rule 111-8-62-.31) that a home accredited by an approved body may substitute that for routine inspection.
- **BSITF:** the Brain and Spinal Injury Trust Fund — flexible, payer-of-last-resort funding for TBI/SCI needs.
- **Level of care:** the assessment that determines whether someone qualifies for waiver services (ICWP requires nursing-facility/hospital level of need).
- **CARF / Joint Commission:** the two national accreditors payers recognize; CARF specializes in brain-injury and rehabilitation programs.

Your Compliance Foundation

Opening the home is the first milestone; staying compliant with Chapter 111-8-62 — and ready for accreditation — is the ongoing work. Two companion resources are built for that: a Policy & Procedure Manual covering the licensure standards and the accreditation framework, and a Forms Packet containing the forms your policies reference. Together they give you the documentation backbone HFRD and an accreditor expect.

Before you rely on any step in this guide

Verify current requirements, forms, fees, and rates with DCH HFRD and, for funding, with the ICWP program and the resident's payer. Georgia has no single brain-injury per-diem, and ICWP has a waiting list — confirm each resident's funding before admission.